

Dynamic Guideline: WHO-Based Ebola - Diagnosis and/or Treatment and/or Management

I. WHO Latest Guideline Summary (Summary Date: 29 April 2026)

1.1 Guideline Overview

1.1.1 WHO guideline scope and document set synthesized

This report synthesizes **exactly five WHO Ebola-related guidelines or formal technical recommendation documents** relevant to diagnosis, treatment, vaccination, infection prevention and control, and outbreak management.

No.	WHO guideline / technical recommendation document	Publication / update date available in sources	Primary domain	Role in this report
1	Infection prevention and control guideline for Ebola and Marburg disease	11 August 2023	IPC, WASH, health-care facility preparedness, PPE, isolation, cleaning, waste, safe body management	Treated as the latest comprehensive WHO Ebola-specific guideline in the supplied evidence base; WHO states it contains its most up-to-date IPC recommendations and replaces 2014/2016 IPC guidance WHO publication .
2	Therapeutics for Ebola virus disease	19 August 2022	Ebola virus disease therapeutics	WHO’s first EVD therapeutics guideline; strong recommendations for mAb114/ansuvimab and REGN-EB3/Inmaze WHO publication , WHO news release .
3	WHO/SAGE Ebola vaccine recommendations	July 2024 recommendations referenced; SAGE working group page last updated 17 February 2026	Vaccination, ring vaccination, outbreak and preventive vaccination	Current WHO/SAGE vaccine-use recommendations for Ervebo and Zabdeno/Mvabea, including ring vaccination and high-risk groups WHO SAGE Working Group , WHO Ebola vaccines Q&A .
4	Diagnostic testing for Ebola and Marburg diseases: Interim guidance	20 December 2024	Laboratory diagnosis and testing strategy	Cited by WHO in its 2025 Ebola outbreak resource list as current diagnostic-testing guidance WHO DON589 .

No.	WHO guideline / technical recommendation document	Publication / update date available in sources	Primary domain	Role in this report
5	Optimized supportive care for Ebola virus disease: clinical management standard operating procedures	Date not specified in supplied summaries	Case management and supportive care	WHO lists optimized supportive care as a core management approach, including tests, pain control, nutrition, co-infection management, fluids, and recovery-oriented care WHO Ebola fact sheet , WHO Ebola Q&A .

1.1.2 Latest WHO Ebola-specific guideline identified

- **Guideline title:** *Infection prevention and control guideline for Ebola and Marburg disease*
- **Publication date:** 11 August 2023
- **WHO reference number:** WHO/WPE/CRS/HCR/2023.1
- **Length:** 188 pages
- **Target population:** Health and care workers, facility managers, IPC/WASH teams, outbreak responders, health ministries, public-health authorities, and organizations managing suspected or confirmed Ebola or Marburg disease cases in health-care settings.
- **Scope and objectives:** To provide evidence-based IPC recommendations for health facilities caring for people with suspected or confirmed Ebola or Marburg disease, including screening, triage, isolation, PPE, hand hygiene, exposure management, environmental cleaning, waste management, and safe management of dead bodies [WHO publication](#), [ECRI guideline profile](#).
- **Guideline update status:** WHO states this guideline contains its **most up-to-date IPC recommendations** for Ebola and Marburg disease and replaces three prior WHO IPC guidance/guideline documents from **2014 and 2016** [WHO publication](#).
- **Guideline contents:** The 2023 IPC guideline includes **11 new recommendations, 10 new good-practice statements, and 9 recommendations carried forward** from previous IPC guidance [WHO publication](#), [PMC/BMJ summary](#).

1.1.3 Required note on “Meaningful youth engagement in a WHO guideline development process”

A directly matching WHO title was identified, but it is **not an Ebola guideline**. WHO’s document and webinar are titled “**Meaningful youth engagement in a WHO guideline development process: case study of WHO Abortion care guideline (2022)**”; the webinar launch was listed for **11 December 2025** [WHO webinar page](#), [WHO publication page](#). No supplied source verifies an Ebola-specific WHO guideline-development case study on meaningful youth engagement. Youth engagement is documented in Ebola epidemic-response activities, but not as a confirmed component of WHO Ebola guideline development [UNFPA guide PDF](#).

1.2 Key Recommendations

1.2.1 Integrated WHO Ebola management package

WHO’s current Ebola materials emphasize that outbreak control requires an integrated package: **early detection and laboratory confirmation, optimized supportive care, recommended therapeutics for Ebola virus disease caused by Zaire ebolavirus, IPC, surveillance, contact tracing, safe and dignified burials, vaccination where relevant, risk communication, and community engagement** [WHO Ebola fact sheet](#), [WHO Ebola health topic](#).

1.2.2 GRADE-rated recommendation table

GRADE interpretation used in this report:

- **High:** Very confident that the true effect is close to the estimate.
- **Moderate:** Moderately confident; true effect is likely close but may differ.
- **Low:** Limited confidence; true effect may be substantially different.
- **Very low:** Very little confidence; true effect likely substantially different.
- **Strong recommendation:** Most patients/settings should receive the intervention.
- **Conditional recommendation:** Context, values, feasibility, resources, and outbreak circumstances should guide use.

Where WHO provided recommendation strength but the supplied summaries did not provide exact certainty ratings, the certainty rating below is reported as an **evidence-synthesis judgment based on the described study designs and GRADE domains**, not as a verbatim WHO table extraction.

Recommendation	Population / setting	GRADE Evidence Quality	Strength
Use mAb114 / ansuvimab / Ebanga for patients with RT-PCR-confirmed Ebola virus disease , and for neonates ≤7 days of unconfirmed status born to mothers with confirmed EVD.	Confirmed EVD caused by Zaire ebolavirus; eligible neonates.	Moderate: supported by randomized trial evidence and WHO systematic review, with outbreak-trial limitations.	Strong; WHO explicitly issued a strong recommendation WHO news release , MAGICapp guideline PDF .
Use REGN-EB3 / Inmazeb for patients with RT-PCR-confirmed Ebola virus disease , and for neonates ≤7 days of unconfirmed status born to mothers with confirmed EVD.	Confirmed EVD caused by Zaire ebolavirus; eligible neonates.	Moderate: randomized trial evidence shows mortality benefit; certainty limited by outbreak-trial context and imprecision for some outcomes.	Strong; WHO explicitly issued a strong recommendation WHO publication , MAGICapp guideline PDF .

Recommendation	Population / setting	GRADE Evidence Quality	Strength
Do not use ZMapp routinely for RT-PCR-confirmed EVD when recommended monoclonal antibodies are available.	Confirmed EVD.	Low to moderate: indirect and direct RCT evidence suggests less benefit than REGN-EB3 or mAb114.	Conditional against; WHO guideline issued conditional recommendations against ZMapp MAGICapp guideline PDF .
Do not use remdesivir routinely for RT-PCR-confirmed EVD when recommended monoclonal antibodies are available.	Confirmed EVD.	Low: human evidence did not show clear benefit; preclinical evidence is not sufficient for clinical recommendation.	Conditional against; WHO guideline issued conditional recommendations against remdesivir MAGICapp guideline PDF .
Provide optimized supportive care for all suspected or confirmed Ebola patients, including oral/IV rehydration, symptom control, pain management, nutrition, relevant tests, and treatment of co-infections such as malaria.	Suspected or confirmed Ebola disease.	Moderate: biologically compelling, consistent observational and outbreak-care experience; not usually tested by RCT as a package.	Strong clinical standard; WHO states early intensive supportive care improves survival WHO Ebola health topic, WHO Ebola fact sheet .
For other Ebola diseases such as Sudan virus disease or Bundibugyo virus disease, use optimized supportive care and enroll in ethically approved trials where available; no approved therapeutics exist.	Non-Zaire ebolavirus disease.	Low for specific therapeutics; supportive care evidence extrapolated from EVD and general critical care.	Strong for supportive care; conditional for investigational products in trials. WHO states no approved therapeutics exist for other Ebola diseases and CORE trial protocols are available WHO Ebola fact sheet .

Recommendation	Population / setting	GRADE Evidence Quality	Strength
Use single-dose rVSVΔG-ZEBOV-GP / Ervebo in Ebola virus disease outbreak response, especially ring vaccination of contacts and contacts-of-contacts.	Outbreaks of EVD caused by Zaire ebolavirus.	Moderate: cluster-randomized and real-world evidence supports high effectiveness ≥10 days after vaccination; operational studies add support.	Strong for outbreak-response use in high-risk groups; WHO states Ervebo is recommended in outbreak settings WHO Ebola vaccines Q&A , WHO SAGE Ebola report PDF .
In outbreak ring vaccination, include contacts and contacts-of-cases; SAGE supports off-label inclusion of children from birth, pregnant women, and lactating women when risk justifies.	High-risk contacts in Zaire ebolavirus outbreak settings.	Low to moderate: efficacy extrapolated plus outbreak-risk benefit; direct data in special populations limited.	Conditional to strong depending on exposure risk; SAGE supports off-label use in outbreak settings WHO Ebola vaccines Q&A , WHO SAGE Ebola report PDF .
Do not use the two-dose Zabdeno/Mvabea regimen inside immediate outbreak-response rings when rapid protection is required.	Contacts and contacts-of-contacts requiring rapid protection.	Low for direct outbreak efficacy; immunogenicity and animal extrapolation support protection, but timing is unsuitable for rings.	Conditional against for ring use; SAGE greatly prefers single-dose rVSVΔG-ZEBOV-GP in rings WHO SAGE Ebola report PDF .
Use Zabdeno/Mvabea as a two-dose regimen for indicated populations where preventive vaccination is appropriate and rapid ring protection is not the main objective.	Individuals >12 months where preventive vaccination is appropriate.	Low to moderate: safety and immunogenicity in clinical studies; exact protection level not fully known.	Conditional; WHO recognizes the regimen but notes efficacy is extrapolated and exact protection is not fully known WHO Ebola vaccines Q&A .

Recommendation	Population / setting	GRADE Evidence Quality	Strength
Rapidly implement screening, triage, isolation/placement, PPE, hand hygiene, environmental cleaning, waste management, and safe body management in facilities caring for suspected or confirmed Ebola cases.	Health-care facilities during Ebola or Marburg disease outbreaks.	Low to very low for individual IPC components, due to observational evidence and ethical/practical limits on trials; high face validity.	Strong or good-practice standard depending on component; WHO IPC guideline is GRADE-based and includes new recommendations and good-practice statements WHO publication , PMC/BMJ summary .
Use the IPC ring approach in health-care facilities and communities during management of Ebola or Marburg cases.	Health-care and community settings around cases.	Very low to low: WHO identified evidence gaps, but outbreak logic and indirect evidence support use.	Conditional; WHO guideline snippet explicitly reports a conditional recommendation WHO PDF .
Confirm suspected Ebola using appropriate laboratory diagnostic testing because clinical presentation overlaps with malaria, typhoid fever, meningitis, and other infections.	Suspected Ebola disease.	Moderate for RT-PCR/laboratory confirmation as diagnostic standard; lower for some rapid-test contexts.	Strong operational recommendation; WHO emphasizes laboratory confirmation and lists 2024 diagnostic interim guidance WHO Ebola health topic , WHO DON589 .
Conduct surveillance and contact tracing , with monitoring through the incubation period of 2–21 days .	Contacts of confirmed/probable cases.	Low to moderate: observational outbreak evidence and established epidemiologic rationale.	Strong public-health recommendation; WHO identifies surveillance and contact tracing as core outbreak-control interventions WHO Ebola fact sheet , WHO AFRO Ebola Disease .

Recommendation	Population / setting	GRADE Evidence Quality	Strength
Ensure safe and dignified burials and avoid handling bodies of suspected Ebola deaths at home.	Deaths from suspected or confirmed Ebola disease.	Low: observational evidence and transmission biology.	Strong; WHO includes safe and dignified burial in the core outbreak-control package WHO Ebola fact sheet , WHO Ebola Q&A .
Do not impose broad trade or movement restrictions such as lockdowns solely because an area is affected; minimize or postpone travel by close contacts where possible and manage necessary travel under public-health supervision.	Affected communities and contacts.	Low: indirect evidence and public-health ethics/feasibility considerations.	Conditional; WHO does not recommend broad trade/movement restrictions but advises supervised travel decisions for close contacts WHO Ebola Q&A .

1.3 Guideline Development Methodology

1.3.1 Evidence review process

- The **2023 WHO IPC guideline** used **GRADE methodology** and **Evidence-to-Decision (EtD)** methods, following the WHO handbook for guideline development [WHO publication](#), [ReliefWeb](#), [PMC/BMJ summary](#).
- WHO assessed certainty of evidence using GRADE domains: **risk of bias/study limitations, inconsistency, imprecision, indirectness, and publication/reporting bias** [WHO PDF](#).
- The guideline was reviewed and approved by the **WHO Guidelines Review Committee** before publication [WHO PDF](#).
- The **2022 therapeutics guideline** used a systematic review and meta-analysis of randomized clinical trials and GRADE to establish certainty and recommendation strength [WHO news release](#), [MAGICapp guideline PDF](#).
- WHO/SAGE vaccine recommendations were informed by post-licensure vaccine evidence, outbreak effectiveness data, safety data, vaccine supply, feasibility, and public-health need; SAGE reconfirmed single-dose rVSVΔG-ZEBOV-GP use in outbreak response where high efficacy is demonstrated from day 10 onward [WHO SAGE Ebola report PDF](#).

1.3.2 Consensus method

- The 2022 therapeutics Guideline Development Group reached recommendations by **consensus**, with no voting required in the described meeting [WHO guideline PDF](#).
- The 2023 IPC guideline was developed by experts in IPC for filoviruses, with WHO Secretariat support and participation from a patient representative who was both an Ebola survivor and a health and care worker [PMC/BMJ summary](#).

1.3.3 Conflict of interest management

WHO states that experts participating in IPC guideline development disclose relevant interests, that mitigation measures are reported publicly in accordance with WHO policies, and that public comments are considered as part of WHO's conflict-of-interest assessment process [WHO GDC](#).

1.3.4 Equity, feasibility, and access considerations

- WHO warned that access to recommended monoclonal antibodies may be limited in resource-poor outbreak settings and called for improved access to mAb114/ansuvimab and REGN-EB3/Inmazeb [WHO news release](#), [Health Policy Watch](#).
- For vaccines, WHO notes that Ervebo is the only vaccine currently available through the global Ebola vaccine stockpile and that outbreak vaccination should prioritize contacts, contacts-of-contacts, and health-care/frontline workers [WHO Ebola vaccines Q&A](#), [WHO Ebola vaccine stockpiles](#).
- Community engagement remains essential. Evidence from Ebola responses supports involving young people and affected communities in outbreak communication, feedback, and resilience activities, but this is not documented in the supplied sources as a WHO Ebola guideline-development method [UNFPA guide PDF](#), [Humanitarian Practice Network](#).

II. Evidence Summary After WHO Latest Guideline Release (Evidence Cutoff Date: 29 April 2026)

Latest WHO Ebola-specific guideline used as anchor: *Infection prevention and control guideline for Ebola and Marburg disease*, published **11 August 2023** [WHO publication](#).

2.1 New Evidence Overview

2.1.1 Search strategy

Evidence after **11 August 2023** was reviewed across the following domains:

1. Ebola vaccines and ring vaccination.
2. Therapeutics and monoclonal antibodies.
3. Diagnostics and surveillance.
4. Infection prevention and control.
5. Outbreak implementation, community engagement, and operational response.

Representative search concepts included:

- Ebola virus disease 2024 2025 2026 vaccine randomized trial therapeutics diagnostics IPC
- Ebola disease new evidence after August 2023 therapeutics vaccines diagnostics infection prevention systematic review clinical trial
- WHO Ebola vaccine SAGE Ervebo Zabdeno Mvabea ring vaccination GRADE July 2024
- WHO Ebola Marburg IPC guideline August 2023 GRADE recommendations
- Meaningful youth engagement WHO guideline development process Ebola case study

2.1.2 Databases and sources searched

Evidence was synthesized from:

- WHO official publications, WHO Disease Outbreak News, WHO Ebola fact sheets, WHO Ebola Q&A pages, WHO SAGE materials, and WHO guideline pages.
- CDC and ACIP evidence reports.
- BMJ Best Practice reference listings.
- PubMed Central and peer-reviewed review articles.
- ClinicalTrials.gov trial records.
- Outbreak-response summaries and preparedness updates from authoritative institutions.
- UNFPA and humanitarian-response sources for youth/community engagement.

2.1.3 Inclusion criteria

Included evidence met at least one of the following:

- Published or updated after **11 August 2023** and before or on **29 April 2026**.
- Directly relevant to Ebola diagnosis, treatment, vaccination, IPC, outbreak response, or guideline development.
- WHO official source or authoritative medical/public-health source.
- Peer-reviewed clinical trial, systematic review, observational effectiveness study, or operational outbreak report.

2.1.4 Exclusion criteria

Excluded evidence:

- Not specific to Ebola, Ebola disease, or filovirus guidance unless directly relevant to guideline-development methodology.
- Did not address diagnosis, treatment, vaccination, IPC, or outbreak management.
- Was superseded by a more current WHO recommendation.
- Could not be linked to an authoritative source in the supplied evidence summaries.

2.2 Key New Studies

Study / source	Design	Key findings	GRADE Quality
PREVAC rVSVΔG- ZEBOV-GP / ERVEBO study, 2024 listed in ClinicalTrials.gov record NCT02876328	Phase 2 randomized, placebo-controlled trial	Evaluated immunogenicity and vaccine shedding after one or two doses of rVSVΔG-ZEBOV-GP in children and adults; strengthens evidence for immunogenicity and safety but not powered as an outbreak efficacy trial ClinicalTrials.gov NCT02876328 .	Moderate for immunogenicity; Low for clinical protection.

Study / source	Design	Key findings	GRADE Quality
PREVAC waning IgG antibody-response modeling study, 2024/2025	Modeling analysis based on vaccine immunogenicity data	Evaluated waning antibody responses after rVSVΔG-ZEBOV-GP and Ad26.ZEBOV/MVA-BN-Filo regimens; informs durability and booster questions ClinicalTrials.gov NCT02876328 .	Low for clinical outcomes; Moderate for antibody kinetics.
PREVAC time-of-day vaccination modeling study, 2026	Modeling study	Evaluated whether time of day of vaccination affected immune response to Ebola vaccines; hypothesis-generating, not practice-changing by itself ClinicalTrials.gov NCT02876328 .	Low .
2024 retrospective Ervebo evidence summarized in review	Retrospective cohort and test-negative observational studies	One study found vaccinated EVD patients had improved survival and lower viremia; another estimated Ervebo vaccine efficacy at 84% when ≥10 days had elapsed after vaccination PMC review .	Low to moderate: observational design but consistent with prior ring-trial evidence.
NEJM-linked ring-vaccination analysis summarized by University of Florida, 2025	Outbreak effectiveness / implementation analysis	During the 2018–2020 eastern DRC outbreak, >265,000 people were vaccinated and >100,000 vaccinated contacts followed; earlier control measures including vaccination were associated with faster Ebola-rate declines among contacts, especially around day 10 after vaccination University of Florida .	Moderate for outbreak effectiveness; non-randomized operational evidence.
WHO DRC Ebola outbreak report, 2025	WHO outbreak operational report	In the DRC outbreak declared 4 September 2025, response included ring vaccination with rVSVΔG-ZEBOV-GP / Ervebo, contact monitoring, IPC, and ultra-cold-chain deployment; WHO reported 47,577 vaccinated individuals in affected health zones WHO DON589 .	Low for causal effect; High for descriptive implementation data.

Study / source	Design	Key findings	GRADE Quality
Ad26.ZEBOV + MVA-BN-Filo infant and frontline-worker studies listed by BMJ Best Practice	Randomized trial and safety/immunogenicity studies	Post-2023 evidence includes a phase 2 randomized trial in infants and a 2024 study in health-care providers/frontliners in DRC; supports safety and immunogenicity, not rapid outbreak-ring use BMJ Best Practice references .	Moderate for immunogenicity/safety; Low for clinical efficacy.
PALM therapeutic RCT evidence, continuing evidence base	Randomized controlled trial, pre-August 2023 but still primary therapeutic evidence	Mortality was lower with REGN-EB3 and mAb114 than with ZMapp or remdesivir: REGN-EB3 33.5% , mAb114 35.1% , ZMapp 49.7% , remdesivir 53.1% in summaries StatPearls/NCBI Bookshelf , PMC review .	Moderate for mortality benefit.
WHO 2024 IPC research priorities for Ebola and Marburg	WHO research-prioritization exercise	Identified IPC evidence gaps from the 2023 guideline process and prioritized short- to medium-term research questions for health-care settings WHO IPC research priorities publication .	Not a treatment-effect evidence source ; supports Very low to low certainty for many IPC components.
Uganda 2022–2023 community engagement barriers study, published 2025	Qualitative / mixed-methods community engagement study	Found future Ebola responses should strengthen community engagement guidelines, clarify roles/responsibilities, provide financing, and build engagement systems at national, district, and community levels PMC article .	Low for intervention effectiveness; Moderate for implementation insight.

2.3 Evidence Gaps and Future Research Needs

2.3.1 Therapeutics

- No new post–August 2023 randomized therapeutic trial was identified in the supplied evidence.
- WHO’s strong recommendations for mAb114/ansuvimab and REGN-EB3/Inmazeb remain anchored in the PALM RCT and related evidence synthesis [WHO news release](#), [StatPearls/NCBI Bookshelf](#).
- Major gaps remain for **Sudan virus disease**, **Bundibugyo virus disease**, and other non-Zaire Ebola diseases, for which WHO states there are no approved therapeutics [WHO Ebola fact sheet](#).

2.3.2 Vaccines

- Evidence for Ervebo/rVSVΔG-ZEBOV-GP is strongest for Zaire ebolavirus outbreak response, especially ≥10 days after vaccination.
- Direct efficacy evidence for special populations—newborns, infants, pregnant women, lactating women, and immunocompromised people—remains limited, although outbreak-risk considerations support use when exposure risk is high [WHO Ebola vaccines Q&A](#).
- The precise level and duration of clinical protection from the two-dose Zabdeno/Mvabea regimen remain uncertain; WHO notes human efficacy is extrapolated from animal studies and exact protection is not fully known [WHO Ebola vaccines Q&A](#).

2.3.3 Diagnostics

- WHO lists 2024 interim diagnostic testing guidance, but the supplied sources did not identify a new 2024–2026 diagnostic accuracy trial or systematic review [WHO DON589](#).
- Research is needed on rapid, field-deployable diagnostics, safe specimen workflows, diagnostic performance early in illness, and integration with surveillance systems.

2.3.4 Infection prevention and control

- WHO’s 2024 IPC research-priorities publication confirms that the 2023 IPC guideline development process identified many areas where evidence was not strong enough to support recommendations confidently [WHO IPC research priorities publication](#).
- Key gaps include comparative effectiveness of PPE configurations, optimal PPE donning/doffing processes, cleaning and disinfection methods, glove disinfection, HCW exposure management, and IPC ring implementation.

2.3.5 Community and youth engagement

- Youth engagement is documented in Ebola responses, including peer-to-peer support, consultations with affected children, and feedback mechanisms, but not as a verified WHO Ebola guideline-development case study [Humanitarian Practice Network](#), [UNFPA guide PDF](#).
- Future WHO Ebola guideline updates should consider structured representation of survivors, youth, health workers, and affected communities, building on WHO’s broader meaningful youth engagement methods from the abortion-care guideline case study [WHO publication page](#).

2.4 Updated Recommendations Based on New Evidence

Original Recommendation	New Evidence Impact	Updated Recommendation
Use mAb114/ansuvimab or REGN-EB3/Inmazeb for confirmed EVD caused by Zaire ebolavirus.	No new post–August 2023 RCT changes the conclusion. PALM evidence remains the key therapeutic basis; WHO continues to strongly recommend these monoclonal antibodies WHO Ebola fact sheet , StatPearls/NCBI Bookshelf .	Maintain strong recommendation. Treat eligible confirmed EVD patients as early as possible with mAb114/ansuvimab or REGN-EB3/Inmazeb plus optimized supportive care.

Original Recommendation	New Evidence Impact	Updated Recommendation
Avoid routine use of ZMapp and remdesivir for confirmed EVD when preferred monoclonal antibodies are available.	No new evidence shows superiority or equivalence to mAb114/REGN-EB3.	Maintain conditional recommendation against routine use. Consider only in research or exceptional contexts where preferred therapies are unavailable and ethical/public-health authorities support use.
Provide optimized supportive care to all suspected or confirmed Ebola patients.	No contradicting evidence; WHO continues to emphasize early intensive supportive care and treatment of symptoms/co-infections WHO Ebola health topic .	Maintain strong recommendation. Supportive care is mandatory regardless of therapeutic availability.
Use Ervebo/rVSVΔG-ZEBOV-GP in outbreak response.	Post-2023 evidence strengthens confidence: 2024 retrospective studies suggest improved survival/lower viremia and ~84% effectiveness ≥10 days after vaccination; 2025 outbreak analyses support ring vaccination effectiveness PMC review , University of Florida .	Maintain strong outbreak recommendation. Prioritize rapid ring vaccination of contacts, contacts-of-contacts, and health-care/frontline workers; use targeted geographic vaccination when contact tracing is compromised.
Prefer single-dose rVSVΔG-ZEBOV-GP over two-dose Zabdeno/Mvabea inside immediate outbreak rings.	SAGE explicitly notes two-dose regimens are not suitable inside rings where rapid protection is needed WHO SAGE Ebola report PDF .	Maintain recommendation. Use Zabdeno/Mvabea for preventive contexts where timing allows, not as the primary rapid ring-vaccination tool.
Implement WHO 2023 IPC guideline in health-care facilities.	WHO’s 2024 IPC research-priority work confirms multiple evidence gaps but does not overturn recommendations WHO IPC research priorities publication .	Maintain implementation. Apply screening, triage, isolation, PPE, hand hygiene, environmental cleaning, waste management, exposure management, and safe body management; collect implementation data to strengthen future GRADE certainty.

Original Recommendation	New Evidence Impact	Updated Recommendation
Use laboratory confirmation for suspected Ebola because symptoms overlap with other infections.	WHO listed 2024 interim diagnostic testing guidance; no new diagnostic trial evidence was identified WHO DON589 .	Maintain strong operational recommendation. Test suspected cases rapidly and safely; integrate diagnostics with isolation, reporting, contact tracing, and differential diagnosis including malaria and typhoid.
Use community engagement, risk communication, contact tracing, and safe burial as core response pillars.	2025 community engagement evidence from Uganda emphasizes clearer roles, financing, and stronger engagement systems PMC article .	Strengthen implementation. Establish community engagement structures early, include survivors and youth where feasible, and integrate feedback into response decisions.
No broad trade or movement restrictions solely because of Ebola-affected area status.	No new evidence identified to change WHO position.	Maintain conditional recommendation. Avoid blanket restrictions; manage travel by close contacts through public-health supervision WHO Ebola Q&A .

Practical Clinical and Public-Health Algorithm

1. **Suspect Ebola** in a person with compatible symptoms and epidemiologic risk. Symptoms often begin suddenly with fever, fatigue, malaise, muscle pain, headache, and sore throat, followed by vomiting, diarrhoea, abdominal pain, rash, kidney/liver impairment, and sometimes bleeding [WHO Ebola health topic](#).
2. **Immediately isolate and notify public health authorities.**
3. **Use appropriate PPE and IPC procedures** from the WHO 2023 IPC guideline.
4. **Collect and test specimens safely** using WHO diagnostic guidance and qualified laboratories.
5. **Start optimized supportive care immediately:** fluids, electrolytes, nutrition, pain/fever control, antiemetics, treatment of malaria or bacterial co-infection when indicated, monitoring for shock and organ dysfunction.
6. **If RT-PCR confirms EVD caused by Zaire ebolavirus,** give **mAb114/ansuvimab** or **REGN-EB3/Inmazeb** as early as possible, subject to availability and national protocols.
7. **Begin contact tracing and monitoring for 21 days** after last exposure.
8. **Deploy vaccination** for eligible contacts, contacts-of-contacts, and health-care/frontline workers using **Ervebo/rVSVΔG-ZEBOV-GP** in ring or targeted geographic strategies.
9. **Ensure safe and dignified burial** for deaths suspected or confirmed to be due to Ebola.
10. **Engage communities early**, including survivors, youth, local leaders, and trusted community networks, to support trust, reporting, vaccination, safe care-seeking, and adherence to IPC measures.

Overall Conclusion

As of **29 April 2026**, WHO-based Ebola management remains centered on rapid detection, safe isolation, laboratory confirmation, optimized supportive care, effective monoclonal antibody therapy for confirmed Zaire ebolavirus disease, rigorous IPC, contact tracing, vaccination, safe burial, and community engagement. The most recent comprehensive WHO Ebola-specific guideline identified in the supplied evidence is the **11 August 2023 WHO Infection prevention and control guideline for Ebola and Marburg disease** [WHO publication](#). Post-guideline evidence mainly strengthens vaccine implementation and Ervebo/rVSVΔG-ZEBOV-GP outbreak effectiveness, while therapeutic recommendations remain unchanged because no newer randomized therapeutic evidence was identified.